

“Protecting Our Children?”

**How Anti-LGBTQ+ Legislation and its Rhetoric Harms the Well-Being of LGBTQ+ Youth
in the U.S.**

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Introduction

I owe a lot to my younger brother, Jupiter, from my music taste to my early understanding of the transgender and nonbinary community. When he was thirteen and I was sixteen, he came out to me as nonbinary. We spent countless hours talking about our identities and mental health, and we built the strongest connection we had ever had as siblings. Jupiter taught me about puberty blockers, testosterone injections, and top surgery as potential medical treatments to make his body more comfortable for him, and I helped educate our parents on using they/them pronouns and gave him feedback on his choices for his new name. After using he/they pronouns for a short time, he told our family “I think it’s just he/him now” and began identifying as a trans guy at fifteen years old.

I am endlessly grateful that we lived in Maryland because the state’s policies allowed Jupiter to access gender-affirming care as a minor – to start testosterone and complete a legal name change at age sixteen. The physical changes he experienced from taking testosterone helped him feel alignment between his body and his internal sense of self; his dysphoria-inducing menstrual periods stopped, and his new facial hair, muscle tone, and deeper voice made him feel like cisgender guys his age, which was his goal. Those physical changes gave him greater confidence, comfort in his body, and – often – gender euphoria. While waiting periods and published notices in local newspapers are usually requirements for minors’ legal name changes, Jupiter’s name change remained private because Maryland exempts gender identity-related name changes from this rule. His name change was completed before he received his driver’s license, and he was able to select the gender marker on his license without any supporting documentation, which protected him from having to out himself to strangers in any situation which required him to show his license. Of course, these triumphs still required the

emotional, legal, and financial support of our parents, but their support and Maryland's policies protected Jupiter.

Even before his legal name change, our teachers respected his pronouns and used his chosen name, and no state or local policy encouraged them not to. Jupiter and I played on a coed high school sports team together, which allowed him to compete in athletics both as a nonbinary person and a trans man, even though no policy segregated our school's sports teams based on sex assigned at birth. Throughout high school, he used a private bathroom which required him to ask a teacher to unlock it each time he needed to use it, not because he was prohibited from using the men's bathroom by anti-trans bathroom bills, but because he did not feel safe doing so alongside the boys who routinely misgendered and misnamed him. Maryland's protective policies did not shelter him from all transphobia, but they facilitated gender-affirming experiences which protected his mental health.

When Jupiter applied to college, he was accepted into universities in Maryland, Pennsylvania, and Virginia. However, my parents and I begged him to stay in Maryland because we trusted the security of transgender rights in the state and, unlike the schools in Pennsylvania and Virginia, the Maryland college was in a heavily liberal region where he could be safely 'out' as a trans man. He did choose the college in Maryland, and he is currently finishing his first year as a civil engineering major with a vast social network and is eagerly counting down the days until his top surgery later this month. I promised him back when we were thirteen and sixteen that I'd be there to help him with his top surgery recovery, and now six years later I will get to fulfill my promise.

It is a privilege that my family's primary concern for Jupiter's care is how far he must travel within the state to find medical practices which (before) provided gender-affirming care

for minors and (now) offer elective top surgery for young adults. However, even though Maryland's laws protect him, the increase in anti-trans sentiment from federal politicians – mirrored at the state level by anti-trans and anti-LGBTQ+ legislation across the U.S. – threatens his safety and challenges his right to equality. The executive orders and federal policies enacted since Donald Trump's second inauguration have curtailed equal treatment of (and data collection on) LGBTQ+ Americans in the organizations under the control of the executive branch, and they have encouraged state governments to do the same (Dawson and Kates 2026).

These attacks on LGBTQ+ rights create uncertainty for Jupiter's future. Will he be able to continue accessing testosterone if federal policies begin to attack gender-affirming care for adults as they already have for minors? Will he face discrimination if he needs to use his birth certificate, his only legal document which does not have his correct name and gender marker, for any reason? However, Jupiter is somewhat protected as an adult; these attacks are even more pressing for minors because many anti-LGBTQ+ bills and policies are specifically intended to restrict their access to healthcare, information, and equal treatment (ACLU 2023b).

Conservative legislators, policymakers, and parents use the rhetoric of 'protecting our children' to promote such anti-LGBTQ+ legislation and policies, yet these measures harm children as they undermine LGBTQ+ youth's mental health and well-being. This project examines the ideological frameworks which motivate conservative Americans to use this 'protection' rhetoric and analyzes the statistical correlation between anti-LGBTQ+ legislation and LGBTQ+ youth's well-being to reveal how unprotecting these measures truly are. By conceptualizing anti-LGBTQ+ legislation as a force minoritizing sexual and gender diversity and as a stressor for LGBTQ+ youth, I interrogate the implications of (failing to) 'protect our children.'

Theoretical Framework

This analysis is framed by the Temporal Intersectional Minority Stress (TIMS) model, an expansion of Ilan Meyer's minority stress theory, which explains how distal (external) and proximal (internal) stressors related to a minoritized sexual identity negatively impact lesbian, gay, or bisexual individuals' mental health. Rivas-Koehl et al. proposed the TIMS model to address limitations of minority stress theory, principally its inability to account for temporal impacts on stress and its exclusive focus on lesbian-, gay-, and bisexual-identified individuals. The TIMS model explains how amalgams of distal stressors like anti-LGBTQ+ discrimination and proximal stressors like fear of prejudice affect the mental health of LGBTQ+ individuals, subject to their intersectional identities and operating within societal hegemonies like cisheteronormativity (Rivas-Koehl et al. 2023). It also considers how current events shape "policies and the social climate for marginalized individuals," and how ontological and generational time impacts individual and community experiences of marginalization (Rivas-Koehl et al. 2023, 716). This framework connects the rhetoric of 'protecting our children' to LGBTQ+ youth's mental health by conceptualizing anti-LGBTQ+ legislation as a distal stressor. Further, because the model incorporates coping and social support factors which can ameliorate the mental health effects of minority stressors, this framework opens this project to the exploration of how LGBTQ+ youth fight back to protect *themselves* (Rivas-Koehl et al. 2023).

Despite the TIMS model's use of the term 'minority stress,' Rivas-Koehl et al. question whether the term "*marginalized stress*" would be more accurate since it acknowledges that "marginalization in society, not one's identity, is responsible for the disproportionate experience of stress" (emphasis in original; Rivas-Koehl et al. 2023, 721). In line with this proposal and with psychologist Phillip Hammack's suggestion that researchers investigate the phenomenon of

marginalization rather than essentializing some sexual or gender experiences as a ‘minority identity,’ I focus on how anti-LGBTQ+ legislation and rhetoric minoritizes those expressions of sexual and gender diversity (Hammack 2025). This stance does not overlook the sense of belonging and personal understanding offered by an identity label or the fact that marginalization is enacted on the basis of identity. Instead, I use the identity-based language of ‘anti-LGBTQ+ legislation’ because it is intended to minoritize particular identities and the language of ‘LGBTQ+ youth’ over ‘sexual and gender minority youth’ to avoid contributing to their marginalization. Further, the mental health survey utilized in this analysis recruited respondents based on sexual and gender identity, so ‘LGBTQ+ youth’ most accurately describes their sexual and gender experiences. This framework therefore centers the process and effects of identity-based marginalization rather than implying that stressors are inherent to specific ‘minority’ identities.

“Protecting Our Children”

In June 2022, members of the conservative group CatholicVote engaged in vigilante censorship of books in library Pride Month displays across the U.S., checking out and refusing to return the books with a publicized mission: “‘To protect our children and the community’” (Loonin 2023, 27). Their declaration implied that exposure to LGBTQ+ characters and stories is harmful to children. In a book meant for young readers, author Meryl Loonin writes that conservatives often condemn books with LGBTQ+ topics as ‘inappropriate’ for children because their content “conflicts with their personal, religious, or political beliefs,” but this does not explain why they conceptualize eliminating children’s access to LGBTQ+ topics as a collective, nationwide necessity rather than a choice to be made only for their own children (Loonin 2023,

31). This position is instead explained by populism, the deeply entrenched cisheteronormativity of conservative political ideology, and the mobilizing power of moral arguments.

Understanding “Protecting Our Children” through the Hungarian Context

Conservative Americans’ rhetoric of ‘protecting our children’ can be partially understood through Hungarian political rhetoric. Soon-to-be-former Hungarian Prime Minister Viktor Orbán’s 16-year-long “electoral autocracy” under the nationalist Fidesz party sought to establish a uniform, conservative Hungarian nation (Kirby 2026; Linnamäki 2022). Orbán defined his government’s policies as grounded in illiberalism, a contrast to the liberal values of other European nations and of the European Union, and he later added Christian values – including anti-LGBTQ+ sentiment – to this ideology. This ideologically established the national Hungarian identity as an explicitly heteronormative identity (Linnamäki 2022).

The Fidesz-endorsed political ideology that family is the most important social institution for (re)producing the nation demanded that the “nationalist, anti-leftist, anti-EU, and Christian stances” of the ruling political party be reflected in the family, which his government explicitly defined in law as a relationship between two heterosexual parents and their children (Linnamäki 2022, 18). Beyond this political mandate of heterosexuality, Fidesz political rhetoric argues that having heterosexual parents and identifying with their sex assigned at birth are imperative for children’s moral and psychological health. Hungarian law also directly links protecting children from non-cisheteronormative influences to protecting them from child abuse offenders, endorsing the false accusation that LGBTQ+ people are pedophiles (Linnamäki 2022). Overall, this rhetoric minoritizes LGBTQ+ people by arguing that exposure to or identification with non-cisheteronormative sexuality or gender harms children’s development.

This background of Hungarian politics is useful because both Hungarian politicians and conservative American politicians endorse nationalist, anti-liberal, Christian stances; consider exposure to non-cisheteronormativity as harmful to children's sexual and moral innocence; and imply that cisheteronormative family constitutes the 'true,' 'moral' nation and that both must be protected from non-heterosexual and non-cisgender influences. However, the ideological foundations of 'protecting children' from LGBTQ+ people and identity (in Hungary, illiberalism, nationalism, and cisheteronormative family; in the U.S., populism and cisheteronormative family) are explicitly named in Hungarian law or by Hungarian politicians, while American politicians leave them implicit. Leaving these ideologies unsaid allows them to operate more covertly, perhaps offering them more power because they are harder to identify and question. However, naming them and explaining their function – as I do below – challenges this silent power.

Why Conservative Americans Use the Rhetoric of “Protecting Our Children”

Populism is the political ideology which discursively divides the nation into the 'moral,' uniform populus and the 'immoral' elite. Populist logic underscores the platforms and rhetoric of American conservative politicians; they claim to speak for the national populus in its entirety and seek to establish themselves as 'one of the people' rather than a socially detached elite (Yett 2024). Their populist rhetoric imagines – and ideologically constructs – a uniform American nation predicated upon shared 'family values,' which establishes the family as comprising the nation (Levine 2002). Whereas Hungarian politicians directly define the family as a cisgender, heterosexual unit with children, American conservatives leave this norm unspoken, yet both marginalize non-cisgender and non-heterosexual experiences. Therefore, under both Hungarian and conservative American logic, to protect children is to protect the family which is to protect

the nation, and protecting the nation requires protecting children and the family (Levine 2002). As they claim to speak for the ‘moral,’ uniform populus, these Hungarian and American politicians position the assumed cisheteronormativity of the family as universal and morally required.

Like in Hungary, conservative American populist logic thus affords cisheteronormativity status as a moral imperative, demanding that ‘moral’ citizens protect vulnerable children from the inappropriate, corrupting influence of sexual and/or gender nonconformity (Yett 2024). Anti-same-sex marriage arguments in the U.S. reflected this demand, relying on the assumption that same-sex marriage legalization would mandate that homosexuality be taught in schools (Oliviero 2018). The consequent argument that curricular instruction would teach “homosexuality and sexuality at developmentally inappropriate times” reflects modern book-banning and curricula-regulating logic of the supposed developmental inappropriateness of non-heteronormative sexuality for minors, as endorsed by Hungarian law (Oliviero 2018, 134). Such arguments also suggest that exposure to ‘adult’ (homo)sexuality “jeopardiz[es] childhood sexual purity,” which, as in Hungary, establishes children’s moral development as predicated on heterosexuality and creates a covert connection to the false allegation that LGBTQ+ people are pedophiles and/or sexual predators (Oliviero 2018, 135). These arguments frame exposure to LGBTQ+ people or identities as a matter of moral right and wrong, which implies that it is thus the duty of all Americans to ‘protect our children’ from non-cisgender and non-heterosexual identity.

Beyond a generic moral call to protect children, this rhetoric of ‘protecting *our* children’ references a notion of children as shared by all members of the moral American nation, making it the duty of all members of the populus to ‘protect’ children from LGBTQ+ identity. By positioning their legal moves such as banning books or prohibiting gender-affirming healthcare

for minors as ‘protecting *our* children’ and families, conservative politicians thus establish this goal as the differentiating characteristic between the ‘moral’ populus – for whom they speak – and the ‘immoral.’

Such populist logic serves to preemptively dismiss opponents as immoral and untrustworthy, and it bolsters voter support, as most voters identify with the imagined moral populus (Yett 2024). Using both an empirical psychological study and a comparison of political party manifestos to voter survey data, political scientist Jae-Hee Jung shows that a political party’s use of moral rhetoric (labeling a matter as a question of right and wrong) increases its members’ positive emotions for the party because they believe the party holds a moral high ground. Voters with positive emotions for their party are more likely to vote in elections, so political parties that use more moral rhetoric tend to have higher voter turnout in elections (Jung 2020). Thus, both in theory and in practice, populist moral rhetoric increases voter support.

This mobilizing effect is not incidental for American conservatives but rather an outcome of carefully curated organizing strategies. For example, in 2008, the Protect Marriage Coalition, an anti-same-sex marriage group, used marketing consultation research to determine that Californian voters were much more likely to oppose same-sex marriage when it was framed as harmful to children. They then implemented this strategy into their marketing across the nation, encouraging organizers to liken the supposed developmental harms to children of exposure to same-sex married couples to those of exposure to pornography in childhood, implying that both equally endanger children’s sexual innocence and again tying LGBTQ+ people to sexual offenses against children (Oliviero 2018). The rhetoric of ‘protecting our children’ is therefore a calculated strategy to increase voter support.

The Rhetoric of “Protecting Our Children” in Legislation

Beyond conservatives’ public statements framing anti-LGBTQ+ legislation as ‘protecting our children,’ such language and sentiments are enshrined in the bills themselves. For instance, in 2023, Missouri enacted the officially named “Missouri Save Adolescents from Experimentation (SAFE) Act,” which prohibits gender-affirming healthcare for minors and imprisoned people and prohibits the use of state healthcare funds to pay for gender-affirming healthcare (S.B. 49). Labeling medical services guided by the standards of care as ‘experimentation’ bolsters the argument that allowing gender nonconformity is a danger to children and is legislators’, policymakers’, and parents’ duty to prevent.

The law’s definition of ‘biological sex’ as, in part, “the biological indication of male or female in the context of reproductive potential or capacity” ties sex to reproduction, which anti-LGBTQ+ Hungarian laws also do (S.B. 49, 1; Linnamäki 2022). This emphasis on reproduction is further reflected in the law’s definition of ‘gender transition surgery’ as including “Surgical procedures that sterilize, including, but not limited to, castration, vasectomy, hysterectomy, oophorectomy, orchiectomy, or penectomy” (S.B. 49, 2). These definitions reference the anti-LGBTQ+ belief that the purpose of sex/sexuality is reproduction and that queer people who do not have children through heterosexual sex and transgender, nonbinary, and gender expansive people who undergo gender-affirming operations which impact their reproductive capacity are failing to fulfil some sort of biological imperative. While directly attacking and marginalizing transgender, nonbinary, and gender expansive people, the act indirectly marginalizes non-heterosexual people as well by reproducing cisheteronormativity and enshrining it as law.

The idea that one’s sex and sexuality must be used for heterosexual reproduction reflects the populist notion that the cisheteronormative family unit conceptually comprises the moral

American nation, adding that the nation is physically reproduced through cisheteronormative family. Family membership is therefore a conceptual requisite for membership in the American nation, so LGBTQ+ people who do not fulfil the cisheteronormative, parents-children familial mandate do not qualify for ‘true’ Americanness (Levine 2002). Through these ideological moves, conservative populist rhetoric marginalizes LGBTQ+ identities.

Beyond these discursive implications of anti-LGBTQ+ legislation and rhetoric, they have material effects as distal stressors for LGBTQ+ youth, impacting their mental and political health. The following statistical analysis investigates the relationship between anti-LGBTQ+ legislation and LGBTQ+ youth’s well-being at the state level.

Data Analysis

Data Sources

The American Civil Liberties Union (ACLU) reports the status (e.g., introduced, passed into law, defeated) and topic (e.g., healthcare restrictions, free speech and expression bans) of all anti-LGBTQ+ legislation proposed in each of the 50 states, Guam, Puerto Rico, the U.S. Virgin Islands, and Washington, D.C. from 2018 to 2026 (ACLU 2023b). The legal staff at the ACLU’s LGBTQ & HIV Project “uses a bill-tracking service” and collaborates with local organizations to identify bills in state legislatures which attack LGBTQ+ rights (ACLU 2023a). The legislation categorized by the ACLU as anti-LGBTQ+ includes free speech and expression bans which are often used to prohibit drag performances, healthcare restrictions for accessing gender-affirming care, public accommodation bans which prohibit transgender people from using bathrooms or locker rooms aligned with their gender identity, bills to weaken LGBTQ+ people’s civil rights, bans on transgender students’ participation in school sports, school curriculum censorship of

LGBTQ+ topics, barriers to accurate identification documents for transgender people, and any other measure to erode LGBTQ+ rights (ACLU 2023b).

It is important to note that the amount of state-level¹ anti-LGBTQ+ legislation introduced and passed is an imperfect measure of the anti-LGBTQ+ policies youth in each state face because local- and school district-level policies also restrict children's access to information and healthcare (Schermele 2022). While citizens of all states are affected by federal policies, state policies or states' refusal to cooperate can somewhat protect their residents from anti-LGBTQ+ federal policies or recommendations. For instance, in April 2025, the Deputy Administrator and Director of the Centers for Medicare & Medicaid Services issued a letter to state Medicaid directors suggesting that they limit or eliminate gender-affirming care for minors from qualifying for Medicaid coverage (Dawson and Kates 2026). The letter implies that states must do so for their Medicaid policies to remain "consistent with applicable law," but it does not change federal law governing Medicaid (CMS 2025, 2). States can thus choose to not comply, meaning some may reject these anti-LGBTQ+ measures while others accede, so residents of different states may not be equally affected by federal anti-LGBTQ+ policy recommendations. Despite and including these nuances, the amount of anti-LGBTQ+ state legislation is an appropriate measure for comparison with other state-level statistics and has significant variance across states, making it a useful factor for data analysis.

The 2024 cycle of The Trevor Project's annual U.S. National Survey on the Mental Health of LGBTQ+ Young People surveyed LGBTQ+ youth between the ages of 13 to 24 in all 50 states, Puerto Rico, and Washington, D.C. from September to December 2023. The published

¹ I use "state" throughout this analysis to refer to all states and territories, not to erase the political status of the territories, but for succinctness.

survey report provides the state-level proportions of respondents who self-reported particular experiences or conditions within the previous year: e.g., considered suicide, experienced depression, or experienced discrimination (Nath 2025). Many anti-LGBTQ+ bills specifically intends to legislate the lives of minors, so this survey captures the experiences of some respondents not directly affected by those bills. However, all such bills, regardless of their target of repression, contribute to an anti-LGBTQ+ political and social atmosphere and thus threaten all LGBTQ+ people and their well-being. Additionally, LGBTQ+ youth surveyed in 2023 may have been minors at the time of the introduction or implementation of some anti-LGBTQ+ bills targeting children and may have experienced the direct consequences of those laws.

A potential limitation of this data source is that The Trevor Project reports the survey results only in state-level aggregate rather than by individual records.² While this is understandably done to protect the privacy of the respondents – especially those under the age of 18 –, it precludes more detailed analysis, including of the proportions of respondents of color, respondents under age 18, or other important subgroups in each state who reported a particular experience or condition. Unfortunately, this is a necessary tradeoff because The Trevor Project’s data is the best national representative survey reporting data of this kind; the Centers for Disease Control and Prevention’s Youth Risk Behavior Survey and Behavioral Risk Factor Surveillance System both collect mental health and sexual identity data and report individual responses, but the former does not include the state in which each respondent resides, the latter only includes adult respondents and does not ask about transgender identity, and neither ask about nonbinary gender identities (CDC n.d.; CDC 2024). Thus, by providing state-specific mental health data for

² The Trevor Project may release the data organized as individual records to qualified researchers, but requesting such access was beyond the timeline of this project.

youth who experience sexual and/or gender marginalization, The Trevor Project’s survey is unique and provides the best data for analyzing American LGBTQ+ youth’s mental health.

Methods: Data Inclusion Criteria and Sample Statistics

This analysis uses only data from the 50 states and Puerto Rico because Guam and the U.S. Virgin Islands were included in the ACLU’s dataset but not The Trevor Project’s survey, and although Washington, D.C. was included in both datasets, its legislative process – and thus its legislative data – is not comparable to those of the 50 states and territories.

Among the survey data from the Trevor Project, this leaves a sample of 18,586 respondents. The number of respondents per state ranged from 64 in Wyoming to 1801 in California, and the median number of respondents was 234 in Connecticut (Nath 2025; see Appendix B for the count of respondents from each state). While the state sample sizes are approximately proportional to the population of each state, they make comparisons among states somewhat less reliable. Since The Trevor Project reports the state-level proportions of respondents who reported a particular experience or condition, states with fewer respondents may have less reliable proportions since smaller sample sizes tend to have higher variation in responses. However, given that the samples are approximately representative, these differences in reliability should be minimal.

The Trevor Project’s survey intended to collect a representative sample “with respect to geography, gender identity, and race/ethnicity,” which it achieved through recruiting respondents via targeted advertising on social media (Nath 2025, 470). Among the overall sample included in this analysis, 49% of respondents were aged 13 to 17, and 51% of respondents were aged 18 to 24. By sexual orientation, the sample was 13% gay, 16% lesbian, 28% bisexual, 16% pansexual,

12% queer, 10% asexual, 1% questioning, and 4% straight or heterosexual. By gender identity, respondents were 33% girls or women; 28% boys or men; 33% nonbinary, bigender, genderfluid, or genderqueer; and 6% questioning. 43% of respondents were cisgender, 53% were transgender or nonbinary, and 6% were questioning whether they were transgender or nonbinary. By race/ethnicity, the sample was 61% white, 12% Hispanic/Latinx, 8% Black or African American, 6% Asian American or Pacific Islander, 1% Native or Indigenous, less than 1% Middle Eastern or North African, and 12% more than one race/ethnicity (Nath 2024).³ Most state sample sizes' demographics closely reflect these overall demographics, suggesting that reported measures are comparable across states (Nath 2025).

This analysis also uses only the legislative data from 2020 to 2023 to capture the anti-LGBTQ+ legislation introduced and passed before The Trevor Project's survey data was collected in late 2023, as these bills were the ones impacting respondents' lives at the time of the survey. Further, given that there is little and poorly-reported data from the ACLU for 2018 and 2019,⁴ these years are excluded from analysis here.

In this analysis, the state-level legislative measures from the ACLU data include the number of anti-LGBTQ+ bills introduced from 2020 to 2023; the number of anti-LGBTQ+ bills passed into law from 2020 to 2023; and the proportion of anti-LGBTQ+ bills passed from 2020 to 2023, calculated by dividing the number passed by the number introduced over that period. The proportion of bills passed offers a comparison of the number introduced and number passed

³ These demographics, as reported in Nath 2024, come from the original sample of 18,663 respondents. However, removing the 63 respondents from Washington, D.C. and the 14 other unaccounted for respondents – who presumably had no state residency data – does not change these demographics. Therefore, they are also accurate for the sample of 18,586 respondents used in this analysis.

⁴ Anti-LGBTQ+ bills became more common beginning in 2020, so the ACLU did not have many bills to track and likely did not need a particularly well-structured reporting format before then.

in one numerical measure. This is useful because it accounts for the differences between two states which, for example, both introduced 13 bills but one passed 8 (Montana) while the other passed 0 (New Jersey) or the differences between two states which both passed 7 bills but one had introduced 16 (Utah) while the other had introduced 71 (Texas). The state-level well-being measures from The Trevor Project survey are separated into two categories. The mental health measures include the proportion of respondents who, in the previous year: considered suicide, experienced anxiety symptoms, and experienced depression symptoms. The political measures include the proportion of respondents who, in the previous year: experienced gender- or sexuality-based discrimination, felt that recent politics affected their well-being “a lot,” and considered relocating to another state due to anti-LGBTQ+ politics. For some of these measures, The Trevor Project separately reports the proportions for all respondents and the proportions for the subpopulation of transgender and gender-nonconforming (TGN) respondents (Nath 2025). Comparing these measures enables analysis of the statistical correlation between anti-LGBTQ+ legislation and LGBTQ+ youth’s mental health.

The number of anti-LGBTQ+ bills introduced in the state legislature from 2020 to 2023 ranges from 0 in New York to 79 in Missouri, and the number of anti-LGBTQ+ bills passed from 2020 to 2023 ranges from 0 in 25 states and Puerto Rico to 20 in Tennessee. These same 26 states and territories had a passing proportion of 0%, while Arkansas reported the highest passing proportion (61.9%), passing 13 of 21 bills (see Appendix A for the number of bills introduced and the number and proportion of bills passed in each state). Of the 935 state-level anti-LGBTQ+ bills introduced from 2020 to 2023, 525 (56%) directly legislated the lives of LGBTQ+ youth. 180 bills (19%) sought to ban transgender youth from sports; 94 bills (10%) looked to limit or eliminate minors’ access to gender-affirming healthcare; and 284 bills (30%) endeavored to

censor LGBTQ+ themes from school curricula, prohibit the use of gender-affirming names and pronouns in schools, and otherwise exclude LGBTQ+ students from equal treatment.⁵ Whether these bills are passed into law or are defeated on the legislature floor, their introduction entrenches an anti-LGBTQ+ atmosphere that threatens LGBTQ+ youth and their well-being.

Results: Mental Health Measures

Simple linear correlation analysis shows that the number of anti-LGBTQ+ bills passed is correlated at an R^2 of 0.18 with the proportion of LGBTQ+ youth who considered suicide within the previous year and at an R^2 of 0.25 with the proportion of TGN youth who considered suicide within the previous year (see Table 1 for all correlations between legislative and mental health measures). This means that the number of anti-LGBTQ+ bills passed accounts for 18% and 25% of the variation in the proportions of LGBTQ+ youth and TGN youth, respectively, who considered suicide in the previous year. There is less correlation between the number of bills passed and the proportions of LGBTQ+ and TGN youth who experienced anxiety symptoms; the R^2 values for the correlations between these measures are from 0.09 and 0.14, respectively, suggesting that 9% of the variation in LGBTQ+ youth's reported anxiety symptoms and 14% of the variation in TGN youth's reported anxiety symptoms can be explained by variation in the number of bills passed. The correlation between the number of anti-LGBTQ+ bills passed and the proportion of respondents who experienced anxiety symptoms in the previous year, like the proportion who considered suicide, is stronger for TGN respondents than for the total sample of LGBTQ+ respondents, suggesting that TGN youth may be more impacted by the marginalizing effect of anti-LGBTQ+ legislation than cisgender youth with minoritized sexualities.

⁵ These legislation counts and proportions do not add up to the total of 525 or 56% because some bills aimed to complete multiple of the listed functions.

Table 1. Correlations between anti-LGBTQ+ legislation measures and LGBTQ+ youth's mental health measures

	LGBTQ+ Suicide Consideration	TGN Suicide Consideration	LGBTQ+ Anxiety	TGN Anxiety	LGBTQ+ Depression	TGN Depression
Bills Introduced	$R^2 = 0.08$ ($p = 0.587$)	$R^2 = 0.15$ ($p = 0.286$)	$R^2 = 0.08$ ($p = 0.569$)	$R^2 = 0.09$ ($p = 0.527$)	$R^2 = 0.06$ ($p = 0.680$)	$R^2 = 0.02$ ($p = 0.873$)
Bills Passed	$R^2 = 0.18$ ($p = 0.206$)	$R^2 = 0.25$ ($p = 0.073$) ⁺	$R^2 = 0.09$ ($p = 0.517$)	$R^2 = 0.14$ ($p = 0.325$)	$R^2 = 0.19$ ($p = 0.180$)	$R^2 = 0.16$ ($p = 0.250$)
Proportion Passed	$R^2 = 0.19$ ($p = 0.176$)	$R^2 = 0.19$ ($p = 0.173$)	$R^2 = 0.09$ ($p = 0.547$)	$R^2 = 0.16$ ($p = 0.260$)	$R^2 = 0.21$ ($p = 0.148$)	$R^2 = 0.19$ ($p = 0.184$)

⁺ marginally statistically significant ($0.05 < p < 0.10$)

The number of and proportion of bills passed is correlated to the proportion of LGBTQ+ youth who experienced depression symptoms at R^2 values of 0.19 and 0.21, respectively, and is correlated to the proportion of TGN youth who experienced depression symptoms at R^2 values of 0.16 and 0.19, respectively. Thus, 16% to 21% of the variation in LGBTQ+ and TGN youth's reported depression symptoms can be explained by variation in the number and proportion of bills passed. The slightly stronger correlation between depression symptoms and the proportion of anti-LGBTQ+ bills passed than the number passed could be explained by higher general anti-LGBTQ+ sentiment in states that pass a high proportion of anti-LGBTQ+ legislation, which may also serve as a distal stressor for LGBTQ+ youth and negatively affect their mental health. General anti-LGBTQ+ sentiment could act as a confounding factor among all these correlations; this sentiment could influence lawmakers to propose and pass more anti-LGBTQ+ legislation and negatively impact LGBTQ+ youth's mental health. These correlations therefore do not signify cause and effect between anti-LGBTQ+ legislation and LGBTQ+ youth's mental health but instead represent a relationship which exists between the measures and which may be affected by other factors like LGBTQ+ acceptance.

Among these correlations with mental health measures, only the correlation between the number of anti-LGBTQ+ bills passed and the proportion of TGN youth who considered suicide in the previous year is marginally statistically significant ($p=0.073$).⁶ Unlike the R^2 values, which are descriptive statistics which describe the correlation that does exist between the variables, the p-values are inferential statistics which describe the probability that the correlation would occur randomly. A high p-value means that a correlation has a high probability of occurring randomly, and a low p-value means that a correlation has a low probability of occurring randomly. The p-value of 0.073 for the correlation between the number of bills passed and the proportion of TGN youth who considered suicide in the previous year means that it has a 7.3% chance of occurring randomly. This suggests that the relationship between the number of bills passed and the proportion of TGN youth who considered suicide in the previous year is a particularly notable finding among the correlations with these mental health measures.

While these percentages (ranging from 9% to 25%) for these correlations seem low, they represent a considerable relationship between anti-LGBTQ+ legislation and LGBTQ+ and TGN youth's consideration of suicide, experience of anxiety symptoms, and experience of depression symptoms given the range of complex factors which impact those mental health experiences. It is notable, however, that the relationship between the mental health measures and the number of bills introduced (ranging from 2% to 15%) is consistently weaker than their relationship with the number or proportion of bills passed. This suggests that the enactment of anti-LGBTQ+ legislation is generally more threatening to LGBTQ+ youth's mental health, or, as indicated above, it is possible that states with both more anti-LGBTQ+ bills passed and poorer mental

⁶ Statistical significance is typically determined by a p-value less than 0.05. However, in the social sciences, p-values between 0.05 and 0.10 are generally considered marginally statistically significant because the traditional cutoff for statistical significance ($p=0.05$) is an arbitrary measure.

health reports for LGBTQ+ youth have a generally higher anti-LGBTQ+ social atmosphere which influences both factors. State legislatures which introduce high numbers of anti-LGBTQ+ bills without passing many of them may simply have a few lawmakers who propose such bills and lack the majority legislature support or the gubernatorial support to pass them. The number and proportion of anti-LGBTQ+ bills passed may therefore more closely reflect a state's general anti-LGBTQ+ sentiment than the number of bills introduced does.

Results: Political Measures

Among the measures relating to politics reported by The Trevor Project, many have statistically significant correlations with the legislative measures (see Table 2 for all correlations between legislative and political measures). The number and proportion of bills passed is correlated to the proportion of LGBTQ+ youth who experienced gender- or sexuality-based discrimination in the previous year at R^2 values of 0.36 ($p=0.009$) and 0.45 ($p=0.001$), respectively. This means that the number and proportion of bills passed accounts for 36% and 45%, respectively, of the variation in the proportion of LGBTQ+ youth who experienced gender- or sexuality-based discrimination. While many of the anti-LGBTQ+ bills themselves discriminate against LGBTQ+ youth by refusing them fair treatment, the anti-LGBTQ+ atmosphere which they reflect and foster may also encourage citizens to discriminate against LGBTQ+ people, perhaps contributing to these high correlations. Of course, a general anti-LGBTQ+ social atmosphere in a state may affect both the amount of anti-LGBTQ+ legislation passed and the marginalization of LGBTQ+ youth through discrimination.

The number of bills introduced is strongly correlated with the proportion of LGBTQ+ youth who reported that recent politics affected their well-being “a lot” ($R^2=0.34$, $p=0.016$); each measure explains 34% of the variation in the other. The fact that the number of bills introduced

Table 2. Correlations between anti-LGBTQ+ legislative measures and LGBTQ+ youth's political measures

	Discrimination	Affected "A Lot" by Politics	LGBTQ+ Relocation Consideration	TGN Relocation Consideration
Bills Introduced	$R^2 = 0.18$ ($p = 0.218$)	$R^2 = 0.34$ ($p = 0.016$)*	$R^2 = 0.62$ ($p = 0.000$)***	$R^2 = 0.60$ ($p = 0.000$)***
Bills Passed	$R^2 = 0.36$ ($p = 0.009$)**	$R^2 = 0.14$ ($p = 0.313$)	$R^2 = 0.65$ ($p = 0.000$)***	$R^2 = 0.61$ ($p = 0.000$)***
Proportion Passed	$R^2 = 0.45$ ($p = 0.001$)***	$R^2 = 0.18$ ($p = 0.210$)	$R^2 = 0.64$ ($p = 0.000$)***	$R^2 = 0.60$ ($p = 0.000$)***
* $0.05 \leq p$ ** $0.01 \leq p$ *** $0.001 \leq p$				

has a much stronger correlation with the proportion of respondents who reported that recent politics affected their well-being "a lot" than the number of bills passed ($R^2=0.14$) and proportion of bills passed ($R^2=0.18$) suggests that anti-LGBTQ+ legislation affects LGBTQ+ youth regardless of whether it is enacted; its introduction alone creates a hostile political environment for LGBTQ+ Americans. While a correlation of 34% seems small given that it describes the relationship between anti-LGBTQ+ legislation and LGBTQ+ youth feeling affected "a lot" by politics, this only describes the relationship with state-level legislation, not local policies or federal executive orders or anti-LGBTQ+ rhetoric from politicians at all levels of the government which also threaten LGBTQ+ equality.

The number of bills introduced, number of bills passed, and proportion of bills passed are all correlated with the proportion of LGBTQ+ youth and TGN youth who considered relocating to another state because of anti-LGBTQ+ politics. The R^2 values for these correlations range from 0.60 to 0.65, and all have a p-value of approximately 0. The fact that all three legislative measures are strongly correlated with the proportion of respondents who considered relocating to another state suggests that the introduction of anti-LGBTQ+ bills is equally as threatening to LGBTQ+ youth's well-being as their enactment.

Limitations

As noted above, this analysis would be stronger if I had access to the individual records for The Trevor Project's survey data because it would enable calculations of the proportion of respondents under age 18, respondents of color, or respondents with both marginalized sexual and gender identities who reported each mental health and political measure of interest. While the small sample sizes of some states may mean that these sub-samples would be too small to provide representative proportions, it would nonetheless be important to evaluate how respondents' full identities – not just their sexual and gender identities – interact with their marginalization by anti-LGBTQ+ legislation.

The age of The Trevor Project's survey data is another limitation; more recent mental health data could reflect the legislative trends of 2024 to 2026, which have seen even greater numbers of anti-LGBTQ+ legislation introduced in state legislatures. In the first four months of 2026, 526 state-level anti-LGBTQ+ bills were introduced, a number which has already surpassed the 510 introduced in all of 2023 (ACLU 2023; ACLU 2026). This more severe anti-LGBTQ+ legislative atmosphere at the state level is compounded by federal anti-LGBTQ+ policies like Donald Trump's anti-LGBTQ+ executive orders (Dawson and Kates 2026). The Trevor Project has completed data collection for the 2025 wave of the U.S. National Survey on the Mental Health of LGBTQ+ Young People but has not yet released the findings (The Trevor Project 2026). The publication of this next survey wave will enable more current analysis of the relationship between anti-LGBTQ+ legislation and LGBTQ+ youth's well-being.

Additionally, an expansion upon this analysis could incorporate a measure of how LGBTQ+-accepting residents of each state are, calculating the correlation between legislative and mental health or political measures while controlling for this acceptance measure. This could

reveal the strength of the correlations when accounting for the variation in LGBTQ+ acceptance between states, which may act as a confounding factor, affecting both the introduction or enactment of anti-LGBTQ+ legislation and LGBTQ+ youth's mental health and political experiences. The direct relationship between anti-LGBTQ+ legislation and LGBTQ+ youth's well-being would therefore be clearer. As is, the analysis of the relationship between the legislative and well-being measures likely also reflects state-to-state variation in LGBTQ+ acceptance, although both the social and legislative atmospheres contribute to the (non)marginalization of LGBTQ+ youth.

Conclusion: LGBTQ+ Youth Protecting *Themselves*

Children must not be infantilized, relegated as fundamentally different from adults. Arguments for children's 'protection' against exposure to sex, sexuality, or (non-cis)gender identity cite them as 'inappropriate' and thus harmful to children's development, but these are all natural aspects of human life, and their rudiments can be taught to children alongside the values of respect, self-knowledge, and privacy which frame them (Levine 2002). Further, social work expert Gisela Konopka argued that among the true requirements for children's healthy development, "the need to participate as citizens" is primary (Levine 2002, 224). Following this, children assert their equal ability as citizens in their activism opposing anti-LGBTQ+ legislation and asserting queer and trans joy.

From 2020 to 2025, only 12% of all proposed state-level anti-LGBTQ+ bills were enacted as law, in part because of LGBTQ+ youth activists' protests. From testifying in state and federal legislature hearings against proposed anti-LGBTQ+ bills to organizing and attending protests demanding LGBTQ+-protective legislation, LGBTQ+ youth assert their civic rights by combating political acts intended to minoritize them (Hanson 2024; Yurcaba 2021). These

activist interventions are incredibly successful in preventing anti-LGBTQ+ bills from passing, and the bills which do pass face lawsuits from social justice organizations to prevent their measures from being implemented, further resisting institutionalized marginalization (ACLU 2023a). Marginalization is not something passively enacted against LGBTQ+ Americans; it is actively constructed through legislation which curtails LGBTQ+ rights and political rhetoric which entrenches cisheteronormativity, and it is actively challenged by LGBTQ+ activists.

LGBTQ+ youth's activism directly attacks the distal stressor of marginalization, and it also serves as a protective measure for their mental health by providing a sense of belonging to a meaningful activist community and a support system of LGBTQ+ peers (Conner et al. 2023). Other coping or protective strategies include engaging in activities which facilitate queer and trans joy because experiencing this joy improves one's mental health. For my brother, Jupiter, these affirming experiences include getting his hair cut at a barbershop, playing from the 'men's tees' on the golf course, and working out (which improves his mental health both because it is exercise and because he experiences it as gender-affirming). He also enjoys answering people's questions about his experiences as a trans man, and his joy of being 'out' and sharing his identity is another rejection of trans marginalization which protects his mental health.

Mental health counseling is another effective strategy for coping with the effects of marginalization. However, of the LGBTQ+ youth who participated in the 2024 wave of The Trevor Project's U.S. National Survey on the Mental Health of LGBTQ+ Young People, 84% wanted mental health counseling, but only half of this proportion were able to access it. Their reasons for being unable to access counseling included fear of sharing their mental health concerns, inability to afford it, not wanting to get their parent or guardian's permission, fear of being outed as LGBTQ+ to others, and fear that they would not be believed or understood (Nath

2024). These limitations to access reveal social and institutional barriers which must be removed to better protect LGBTQ+ youth from marginalization. However, while there is still much work to be done to truly legally and socially protect our queer and trans children, their activism and expression of queer and trans joy proves that LGBTQ+ youth fight back to protect *themselves*.

Appendix A

Anti-LGBTQ+ bills introduced and passed per state, 2020-2023 (ACLU 2023)

State	Bills Introduced	Bills Passed	Proportion Passed
Alabama	20	5	0.250
Alaska	7	0	0.000
Arizona	29	4	0.138
Arkansas	21	13	0.619
California	1	0	0.000
Colorado	4	0	0.000
Connecticut	4	0	0.000
Delaware	2	0	0.000
Florida	21	6	0.286
Georgia	18	1	0.056
Hawaii	7	0	0.000
Idaho	12	5	0.417
Illinois	2	0	0.000
Indiana	34	4	0.118
Iowa	56	4	0.071
Kansas	21	3	0.143
Kentucky	29	4	0.138
Louisiana	12	2	0.167
Maine	3	0	0.000
Maryland	3	0	0.000
Massachusetts	4	0	0.000
Michigan	14	0	0.000
Minnesota	18	0	0.000
Mississippi	37	3	0.081
Missouri	79	2	0.025
Montana	13	8	0.615

State	Bills Introduced	Bills Passed	Proportion Passed
Nebraska	4	1	0.250
Nevada	2	0	0.000
New Hampshire	11	1	0.091
New Jersey	13	0	0.000
New Mexico	3	0	0.000
New York	0	0	0.000
North Carolina	15	5	0.333
North Dakota	19	11	0.579
Ohio	17	0	0.000
Oklahoma	58	7	0.121
Oregon	7	0	0.000
Pennsylvania	8	0	0.000
Puerto Rico	2	0	0.000
Rhode Island	9	0	0.000
South Carolina	41	2	0.049
South Dakota	14	3	0.214
Tennessee	62	20	0.323
Texas	71	7	0.099
Utah	16	7	0.438
Vermont	2	0	0.000
Virginia	17	0	0.000
Washington	3	0	0.000
West Virginia	31	3	0.097
Wisconsin	29	0	0.000
Wyoming	9	1	0.111

Appendix B

Proportion of respondents who reported each mental health and political measure on the 2024 U.S. National Survey on the Mental Health of LGBTQ+ Young People (Nath 2025)

State	LGBTQ+ Suicide Consideration	TGN Suicide Consideration	LGBTQ+ Anxiety	TGN Anxiety	LGBTQ+ Depression	TGN Depression	Discrimination	Affected “A Lot” by Politics	LGBTQ+ Relocation Consideration	TGN Relocation Consideration	Count of Respondents
Alabama	38%	41%	72%	77%	56%	60%	65%	55%	39%	33%	214
Alaska	44%	51%	75%	75%	61%	69%	57%	60%	43%	47%	83
Arizona	39%	41%	64%	67%	55%	60%	60%	54%	44%	51%	421
Arkansas	45%	53%	68%	74%	52%	64%	66%	57%	60%	67%	166
California	35%	39%	64%	69%	52%	57%	56%	47%	16%	18%	1801
Colorado	41%	47%	68%	73%	56%	62%	61%	52%	26%	29%	450
Connecticut	26%	36%	57%	64%	39%	49%	53%	46%	19%	23%	234
Delaware	37%	45%	67%	69%	52%	64%	58%	41%	29%	42%	84
Florida	37%	45%	67%	70%	54%	59%	63%	60%	69%	78%	984
Georgia	38%	42%	67%	71%	53%	58%	59%	54%	48%	61%	541
Hawaii	32%	33%	74%	82%	43%	61%	50%	46%	19%	24%	68
Idaho	38%	43%	68%	74%	63%	72%	71%	57%	59%	72%	155
Illinois	41%	48%	64%	68%	49%	57%	60%	53%	20%	24%	626
Indiana	43%	46%	65%	68%	53%	59%	63%	57%	51%	60%	424
Iowa	34%	42%	64%	72%	47%	56%	54%	62%	51%	69%	201
Kansas	39%	40%	65%	68%	49%	51%	63%	53%	47%	56%	192
Kentucky	43%	50%	66%	71%	56%	61%	62%	64%	56%	67%	237
Louisiana	32%	42%	65%	75%	48%	64%	62%	49%	56%	67%	191
Maine	38%	42%	71%	72%	50%	55%	59%	46%	18%	20%	116
Maryland	40%	44%	57%	61%	48%	54%	56%	43%	20%	24%	339
Massachusetts	39%	47%	68%	73%	50%	58%	54%	53%	12%	14%	454
Michigan	37%	42%	68%	70%	52%	57%	59%	52%	29%	35%	624
Minnesota	39%	39%	64%	68%	52%	55%	57%	49%	18%	23%	421
Mississippi	37%	41%	72%	79%	59%	61%	63%	51%	52%	63%	105
Missouri	39%	45%	69%	74%	49%	57%	61%	61%	61%	67%	408
Montana	44%	49%	66%	65%	56%	57%	64%	56%	53%	61%	113
Nebraska	45%	47%	69%	73%	57%	53%	67%	68%	61%	73%	113
Nevada	45%	52%	66%	71%	60%	64%	60%	52%	26%	28%	145
New Hampshire	36%	40%	64%	68%	45%	44%	65%	55%	31%	31%	122
New Jersey	34%	41%	62%	67%	48%	53%	57%	47%	16%	20%	404
New Mexico	43%	44%	64%	66%	56%	62%	71%	59%	30%	36%	121

State	LGBTQ+ Suicide Consideration	TGN Suicide Consideration	LGBTQ+ Anxiety	TGN Anxiety	LGBTQ+ Depression	TGN Depression	Discrimination	Affected “ A Lot” by Politics	LGBTQ+ Relocation Consideration	TGN Relocation Consideration	Count of Respondents
New York	39%	45%	64%	71%	48%	52%	59%	46%	18%	22%	900
North Carolina	41%	47%	66%	70%	52%	57%	63%	57%	48%	56%	550
North Dakota	36%	41%	66%	80%	53%	66%	57%	41%	62%	68%	80
Ohio	43%	50%	68%	73%	56%	61%	61%	56%	52%	61%	702
Oklahoma	46%	52%	69%	70%	58%	61%	66%	57%	68%	73%	221
Oregon	41%	45%	66%	72%	57%	62%	61%	56%	19%	23%	329
Pennsylvania	36%	41%	69%	74%	53%	60%	58%	50%	27%	34%	653
Puerto Rico	40%	39%	65%	67%	55%	61%	51%	36%	24%	33%	131
Rhode Island	37%	49%	67%	70%	51%	52%	65%	57%	16%	19%	107
South Carolina	40%	44%	66%	70%	53%	61%	63%	55%	55%	62%	233
South Dakota	44%	47%	67%	75%	52%	58%	56%	49%	52%	64%	95
Tennessee	42%	50%	69%	73%	57%	59%	64%	52%	62%	70%	395
Texas	42%	49%	66%	70%	55%	60%	60%	54%	58%	66%	1490
Utah	42%	46%	66%	69%	53%	56%	66%	56%	56%	63%	381
Vermont	44%	50%	69%	69%	58%	62%	59%	56%	20%	28%	72
Virginia	42%	48%	64%	72%	50%	57%	55%	55%	39%	46%	522
Washington	37%	42%	66%	71%	52%	59%	61%	57%	23%	26%	625
West Virginia	40%	51%	74%	75%	58%	69%	62%	58%	46%	55%	121
Wisconsin	39%	44%	63%	68%	50%	54%	61%	53%	40%	45%	358
Wyoming	56%	60%	83%	88%	70%	76%	62%	53%	59%	72%	64

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